EXECUTIVE BRIEFING | POLICY & OPERATIONS OVERSIGHT

System Capacity & Care Load Analytics

Inter-Agency Unaccompanied Children (UAC) Care Pipeline Assessment (2023–2025)

Purpose: To brief senior leadership at the Department of Health and Human Services (HHS) and Customs and Border Protection (CBP) on data-driven metrics for evaluating active care load, detecting inter-agency bottlenecks, and stabilizing the UAC care delivery pipeline during high-volume periods.

KEY PERFORMANCE INDICATORS (HISTORICAL AVERAGES)

TOTAL SYSTEM LOAD
11,090
Active children under federal care daily (CBP + HHS)

NET PRESSURE
+25 / day
Average daily intake surplus into HHS facilities

DISCHARGE OFFSET
0.935
Ratio of sponsor releases to incoming transfers

LOAD VOLATILITY
1,850
Standard deviation in active care load across 2023–2025

CORE FINDINGS & SYSTEMIC RISKS

An analysis of daily operational time-series data reveals that capacity strain in the UAC program is primarily driven by **transfer-discharge timing asymmetry** rather than border apprehension rates alone. While CBP holding capacity responds dynamically to intake surges, HHS sponsor vetting workflows require longer operational lead times, generating persistent care load backlogs.

Operational Stage	Primary Vulnerability	Measured Impact	Strategic Risk
CBP Triage & Custody	Rapid intake accumulation during surge events	Peak holding spikes up to 4,850 children	Facility overcrowding & extended holding times
Inter-Agency Transfer	Bed allocation lag between CBP & HHS	Daily transfers average 385 (peak 740)	Logistical delays in medical screening & shelter placement
HHS Shelter Care	High baseline occupancy consuming fixed bed quotas	Average occupancy of 8,950 beds (peak 13,200)	Reliance on costly emergency intake sites (EIS)
Sponsor Discharge	Complex vetting processes creating release bottlenecks	Offset ratio drops below 0.65 during surges	Compounding cumulative backlog across shelter networks

OPERATIONAL FLOW & CAPACITY DYNAMICS

During steady-state operations, the Discharge Offset Ratio (R_t) remains close to 1.0, signifying that sponsor reunifications successfully offset incoming transfers from CBP. However, during surge windows, R_t frequently drops below 0.65. Without predictive intervention, a sustained offset deficit of 100 children per day results in an additional 700 shelter beds required each week, rapidly depleting standard ORR reserve bed capacities.

STRATEGIC ACTION PLAN & RECOMMENDATIONS

To transition federal care management from reactive emergency response to proactive capacity governance, the following four strategic interventions are recommended for implementation:

- **1. Operationalize Automated Surge Triggers:** Mandate the implementation of early-warning alerts based on 7-day rolling Net Intake Pressure. When net pressure exceeds +50 children/day for three consecutive days, activate soft-surge shelter staffing protocols before baseline capacities reach 90% utilization.
- **2. Tiered Sponsor Vetting Framework:** Establish streamlined processing protocols for Category 1 sponsors (parents and legal guardians) during high-load periods. Accelerating low-risk reunifications elevates the Discharge Offset Ratio without compromising child safety standards.
- **3. Standardize Live Analytics Adoption:** Deploy the developed Streamlit web dashboard across both CBP and HHS operational coordination centers. Unifying daily metrics ensures equal situational awareness across both agencies and accelerates transfer authorization workflows.
- **4. Regional Capacity Reserves:** Maintain flexible, scalable bed reserves in key geographical corridors to absorb localized border influxes, avoiding forced transcontinental transfers and system-wide pipeline congestion.

CONCLUSION & NEXT STEPS

By implementing continuous capacity monitoring and optimizing sponsor discharge workflows, HHS and CBP leadership can establish a resilient, policy-compliant healthcare and child welfare pipeline that ensures safe, humane, and timely care delivery for all unaccompanied children.

Report Prepared By: Care Load Analytics Working Group **Distribution:** Executive Leadership (HHS / CBP)